



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : نسمة حسن رجب درويش
 المحافظة : القاهرة
 الحي / المدينة : أخرى
 رقم البطاقة : 25612040103805 رقم التليفون : 01005875696
 العمر (تاريخ الميلاد) : 04-12-1956 المهنة : ربه منزل
 العنوان : ممممممممممممممممم
 تاريخ تسجيل الإستمارة : 27-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
 الإخصائي الاجتماعي
 التوقيع: (-----)
 المقر بما فيه
 التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام أشعه مقطعيه علي الصدر بالصبغه [10851] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
 الاسم: د/ندى حسن
 الاسم: د/نرمين رمضان
 الوظيفة: -----
 الوظيفة: -----
 الوظيفة: -----
 التوقيع: -----
 التوقيع: -----
 التوقيع: -----
 أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
 تعتمد صحة البيانات المرفقة
 رئيس اللجنة ومدير
 المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
 خطة العلاج : -----
 المستشفى : -----
 قسم : -----
 نفقات : -----
 جهة العلاج : -----
 ملاحظات : -----

ختم
 شعار الجمهورية

مدير عام
 مديرية الشؤون الصحية

رئيس
 المجلس الطبي المتخصص

--

Name: نسمة حسن رجب درويش National ID: 25612040103805	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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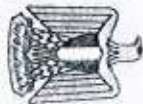
وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced chest CT

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



التي مسلمة متروجة
محمد ابراهيم محمد المدرش
٢٠٣٢/٠١/١١ البطاقة سارية حتى



تسميه
حسن رجب درویش
۱۲ ش محمد الباشا
المعالي - القاهرة



KY2679611

207.12.3.1.38.0

MDT FORM

18/17/2025 G

Name	كريحة مختار علي	MRN	2330
		ID	23
Age	51.0	Phone	01552375069
Menopausal status	Postmenopausal	Marital status	متزوج
NRS SCORE		Family history	-ve of ovarian cancer
BMI		REASON OF MDT	
CO morbidity	Hx of Lt. MRM post NACTH. now on hormone therapy.		

Clinical Data

BX:

ER:

PR:

HER 2:

Ki67:

R

L

Sonomammography:

TVUB →



Mets Workup:

- Lt. ovarian dermoid cyst 2.2 x 1.8 cm.
- The endometrial thickness is 0.5 cm & minimal fluid seen trapped at the cavity.

Recommendations:

RemDT

For laparoscopic bilateral oophorectomy as therapeutic and diagnostic.

*after mri small fibroid 1.1 cm. CCC → for lap chole

MDT OUTCOME & PLAN

MRI abd and pelvis with contrast for lap chole

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

Psychiatric

Handwritten signatures in the signature boxes.

Patient Name : الأستاذ / كريمه مختار على	Request No. : 13006025
Patient No. : 27412122601607	Request Date : 25/06/2023
Path No. : SR-741/23	Verification Date : 26/06/2023
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 48 Y	Department : Out Patient
Referreing Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data:

Left breast mass & Left axillary LN.
Nature of the mass:

Nature of the specimen:

Slide revision

Gross Examination:

1 Block (202860/23) +5 Slides .

Microscopic examination:
Left breast:

Left breast mass :

- Histological type: Invasive duct carcinoma with focal mucinous features
- Grade (differentiation + nuclear pleomorphism + mitosis): G II (3+2+2)
- Angioinvasion: Couldn't be detected in sections examined.
- Ductal carcinoma in situ: No
- IPT:

ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 60% of tumor cells (Allred score 8/8)
Her-2: strong complete

PR: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
Her-2: strong complete membrane reaction in 60% of tumor cells (Allred score 7/8)
FNAC from Lt axillary LN

Her-2: strong complete membrane reaction in >10% of tumor cells (score 3+)

Positive on

Positive smears consistent with metastatic adenocarcinoma.

Diagnosis:

Left breast mass & Left axillary LN, Slide revision :
 Lt breast mass :
 • Invasive

Lt breast mass :

- Invasive duct carcinoma grade II with focal mucinous features
 - ER : Positive (8/8)
 - PR : Positive (7/8)
 - HER2 : Positive (score3+)
- FNAC from left axillary LN :

Verified by :

dr.esraa magdy

Printing Date : 19/07/2023 10:01:27

Page 1 of 2

Printing By : .marwa mohamed youss

Patient Name : الأستاذة / كريمه مختار على
Patient No. : 27412122601607
Path No. : SR-741/23
Sex : Female
Age : 48 Y
Referreing Doc :

Request No. : 13006025
Request Date : 25/06/2023
Verification Date : 26/06/2023
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : .marwa mohamed youss

Pathology Report

Positive smears consistent with metastatic adenocarcinoma .

Prof.Dr.Manal Elmahdy
Dr.Israa Elgendy

Verified by :
dr.esraa magdy

Printing Date : 19/07/2023 10:01:27

Page 2 of 2

Printing By : .marwa mohamed youss



Patient Name: Karima Mokhtar Ali Badr	Date : 09/11/2023
Exam : ultrasound BREAST (BOTH SIDE)	Age : 49 Year(s)
Medical ID: 2330/2023	Pat. ID: 43850

FOLLOW UP BILATERAL BREAST ULTRASOUND

INDICATION:

- Follow up post chemotherapy for left breast neoplastic process.
- In correlation to the previous study of 30/5/2023

REPORT:

Regressive course as regards the size of the previously detected:

- Left breast UIQ (11 o'clock) spiculated hypoechoic mass, currently measuring 1.7x2.6cm (was 2.2x5.6cm in the previous study), showing regressive course (the previously reported calcifications reaching the nipple are to be considered).
- No Left pathological axillary LNs
- No supra clavicular LNs seen
- No right breast suspicious mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Right non specific axillary lymph nodes.

OPINION:

- Follow up post chemotherapy for a case of left breast malignancy, showing partial radiological response (BIRADS 6).
- Signature Dr. Engy Abdallah
- 09/11/2023 3:41 Pm

PATIENT NAME KARIMA MOKHTAR ALI BADR	PATIENT ID #: 20223000562
GENDER: Female	ORDER #: Z8129,Z8130
AGE: 48 y	DATE OF SERVICE: 30-May-2023 01:28 PM

Report: - all studies

**FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY
ULTRASOUND**

INDICATION:

- Palpable left breast lump, of 1 month duration.

TECHNIQUE:

- CC and MLO views of both breasts.
- Complementary ultrasound

REPORT:

- Bilateral heterogeneously dense breasts (ACR C).
- Left breast upper and lower inner quadrant, grouped, pleomorphic microcalcification.
- Complementary Ultrasound showed an irregular, ill-defined, hypoechoic mass lesion that shows possible intraductal extension and considerable vascularity on color Doppler interrogation, seen centered over 11 o'clock location of the left breast (site of clinically palpable lump). It measures approximately 5.6X2.2 cm along its maximal cross sectional dimensions. It shows echogenic calcifications within.
- An area of altered parenchymal echogenicity is seen centered over 9 o'clock location of the left breast, with echogenic calcifications noted within, is seen measuring approximately 4X1.4 cm along its maximal cross sectional dimensions. Mild diffuse thickening of the overlying skin is noted (reaching a thickness of 5 mm).
- Multiple, variable-sized, anechoic, avascular simple cysts are seen scattered within both breasts.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- A small, left axillary lymph node (level I) appears globular (approximately 0.5 cm along its maximal diameter) and muffled hilum.
- Right axillary lymph nodes show average cortical thickness and preserved hila.
- No ultrasonographic evidence of suspicious supra or infraclavicular lymphadenopathy.

OPINION:

- Left breast infiltrative malignant-looking process, with suspicious microcalcification and indeterminate ipsilateral axillary node, as above described, for histopathological verification (BIRADS 5).
- Bilateral fibrocystic changes.

Recommendation - all studies

Referral to clinic

Reported By,

Dr. DALIA EL-MESIEDY, MD

30-May-2023 03:14 PM

PATIENT NAME: **KARIMA MOKHTAR ALI BADR**
GENDER: **Female**
AGE: **48 y**

PATIENT ID #: **20223000562**
ORDER #: **2333744,2333745**
DATE OF SERVICE: **10-Jun-2023 12:38 PM**

Report: - all studies

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

1-CHEST

- Left breast UIQ irregular enhancing soft tissue mass lesion is seen measuring about 2.2 x 4.3 cm associated with mild skin thickening.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Left LL para-mediastinal benign calcific nodule.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion seen.
- Normal cardiac size.
- No pericardial effusion detected.
- Left thyroid lobe tiny hypodense nodule.

2- ABDOMEN AND PELVIS

- The liver is of average size with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Calcular gall bladder.
- Normal appearance of the pancreas, spleen, kidneys, adrenals, aorta, I. V.C and the pelvic organs.
- Under-filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic collections.
- Bulky uterus with heterogenous enhancement and left uterine wall well defined hypodense lesion measuring about 2 x 2.2 cm (? Degenerating fibroid). Left adnexal cysts are also noted, largest is measuring about 2.2 x 4 cm (for clinical correlation +/- local U/S assessment).

The revised bone window revealed:

- 1. No worrisome lytic or sclerotic lesions.
- 2. No CT evidence of vertebral collapse.

Opinion:

- Left breast UIQ malignant-looking mass lesion as described.
- No definite pulmonary, hepatic or osseous metastases.

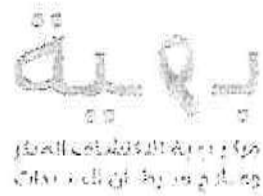
Reported By,

Dr, Peter Fahmy, MD, FRCR

Dr. Fifi Sami, MSc

10-Jun-2023 08:06 PM

قسم الأشعة النووية



PATIENT NAME: KARIMA MOKHTAR ALI BADR	PATIENT ID #: 20223000562
GENDER: Female	ORDER #: 2333746
AGE: 48 y	DATE OF SERVICE: 21-Jun-2023 09:18 AM

Report:

99mTc-MDP BONE SCAN

Desired Information: - skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 minutes & 2 hours after I.V administration of 20 mCi **FINDINGS:-**

Degenerative changes at dorso- lumbar vertebrae, more at lower dorsal for follow up.

Arthritic changes at knees and shoulders.

Otherwise, no hot or cold spots that would account for serious bone lesions.

Both kidneys are seen.

Conclusion: -

No definite evidence of metastatic osseous deposits.

Reported By,

Ass. Prof. Huda Fathy Elsayed

22-Jun-2023 03:04 PM



X - Ray	UltraSound	Doppler	Digital Rad.	C.T.	MRI	Mammography
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Patient Name:	KARIMA MOKHTAR ALI	ID:	
Ref. Phys:		Date:	09/12/2025

MRI OF THE ABDOMEN AND PELVIS :-

The study was performed on a 1.5 Tesla super conducting system.

TECHNIQUE: -

Variable pulse sequences were taken with contrast.

FINDINGS:

- Gall bladder distended with multiple stones.
- Left uterine wall well-defined hypointense small lesion measures about 1 x 1 cm.
- The liver is of normal MRI signal pattern, average size with no focal lesions.
- No intra or extra-hepatic biliary dilatation.
- Normal MRI signal of the spleen with no focal lesions.
- Pancreas is of normal MRI appearance.
- Normal MRI appearance of both kidneys, that show preserved parenchyma with no evidence of focal lesions, no stones or back pressure changes elicited.
- No ascites.
- Normal signal void flow of the major abdominal vessels.
- Normal MRI appearance of the urinary bladder and both ovaries.

OPINION: -

- Calcular gall bladder.
- Left uterine wall fibroid.
- No adnexal mass lesion.

Much Obliged
Dr. Amr El Sakka M.Sc.

M.S

Al - Hussein Hospital - El Darasa - Cairo
Tel : 25105628 - 25109140 Fax: 25104146

Radiology Department

مستشفى الحسين الجامعي - الدراسة - القاهرة
تليفون: ٢٥١٠٥٦٢٨ - ٢٥١٠٩١٤٠ فاكس: ٢٥١٠٤١٤٦